PSYCHIATRIC EMERGENCIES

BMRC FA26

What is a Psychiatric Emergency?

Behavioral emergencies:

Abnormal reaction to a situation without structural brain disease

- Prioritize de-escalation and emotional support

Psychiatric emergencies:

Abnormal behavior caused by underlying psychiatric illness

- May be a threat to patient or bystanders
- Could require a 5150 hold and transport

Terminology

Hallucinations
False sensory perception

Delusions
Fixed false belief

Incoherence
Disorganized speech
Aphasia vs
Dysphasia

Catatonia
Motor immobility or unresponsiveness

Psychosis
Loss of contact with reality
Often delusions and hallucinations

Delirium
Acute, rapid onset of confusion

Physical Causes of AMS (check first)

Head/Brain Trauma:

Unequal pupils, confusion, combativeness, post-concussive AMS.

Hypoglycemia (<60 mg/dL):

Diaphoresis, rapid onset confusion, agitation, drunkenness

Hypoxia:

Restlessness, cyanosis, altered LOC, uncharacteristic aggression

Stroke:

FAST, focal neurological deficits, delirium

Physical Causes of AMS cont.

Substance toxicity or withdrawal:

- Clumsiness, slurred speech, hallucinations, ALOC, respiratory changes

Environmental / Hypothermia:

- Temperature regulation failure, lethargy, delirium

Severe situational stress:

- Hyperventilation, disorientation, acute emotional shock
- Behavioral emergency may progress in acuity

Common Psychiatric Disorders

Depression: Persistent sadness/hopelessness; S/Sx: apathy, suicidal ideation

Anxiety: Overwhelming panic response; S/Sx: tachycardia, hyperventilation

Schizophrenia: Psychotic disorder; S/Sx: hallucinations, disorganized speech

Bipolar Disorder: Mood instability; S/Sx: alternates between major lows and highs

Paranoia: Extreme irrational distrust; S/Sx: hypervigilance, hostility, fear

Alzheimer's: Neurodegenerative disease; S/Sx: severe memory loss, disorientation

Dementia: General cognitive decline; S/Sx: impaired memory, judgment, reasoning

Common Psychiatric Disorders cont.

Dissociative Identity Disorder: Identity fragmentation; S/Sx: distinct personalities, memory gaps

Functional Disorders: Only neurological symptoms; S/Sx: conversion disorder, non-epileptic seizures

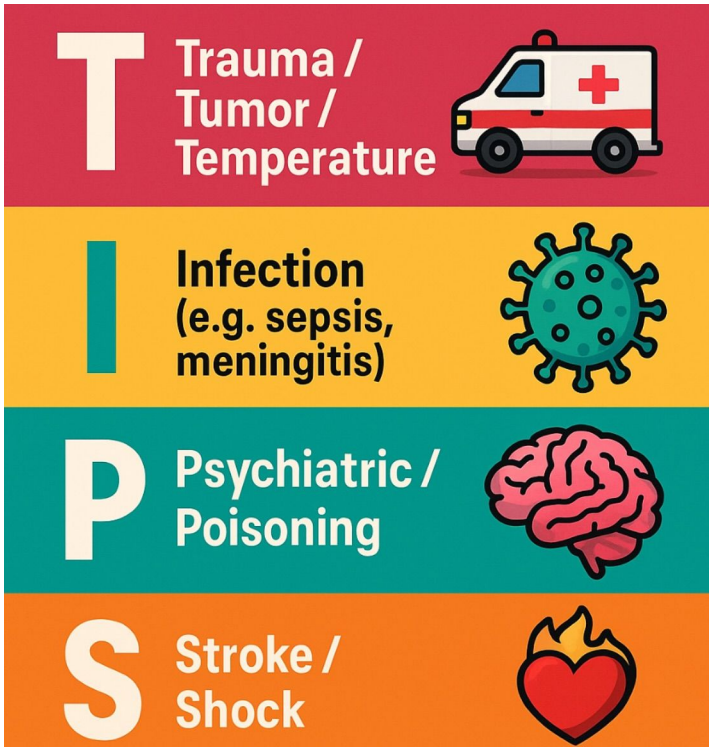
Organic Brain Syndromes: AMS from physical/physiological pathology; S/Sx: acute confusion, altered LOC

Mania: Abnormally elevated mood/energy; S/Sx: eminence, insomnia, impulsivity

PTSD: Trauma-induced stress response; S/Sx: intrusive flashbacks, hyperarousal, acute severe agitation

Mnemonics!


A
Alcohol / Acidosis
E
Epilepsy / Electrolytes / Encephalopathy
I
Insulin (hypo/hyperglycemia)
O
Overdose / Oxygen deficiency
U
Uremia (renal failure)


T
Trauma / Tumor / Temperature
I
Infection (e.g. sepsis, meningitis)
P
Psychiatric / Poisoning
S
Stroke / Shock

Implied Consent

Applies when a patient has ALOC or is under strong effects of a mentally altering substance

Take PPTE, GCS

Look for alcohol and drug markers

Best judgement: is immediate treatment/transport required?

Document

ALCO Release Protocol (pg. 120)

5. **ASSESS AND RELEASE (AAR) FROM EMS CARE CRITERIA:**

5.1 **EMS clinician and the patient or DDM agree that the illness/injury does not require immediate treatment/ transport via emergency/911 services**

5.2 In order to release care, a patient, parent, or guardian must have legal and mental decision-making capacity by meeting all of the following criteria:

5.2.1 Understands the nature of the medical condition, and the risks and consequences of not seeking treatment now

5.2.2 Exhibits evidence of decision-making capacity sufficient to understand the nature of the medical condition as well as the risks and potential consequences of not seeking additional medical care/ transport

5.2.3 Exhibits no evidence of:

- ► Altered level of consciousness
- ► Alcohol or drug ingestion that impairs decision-making capacity

5.3 EMS personnel should advise the patient/DDM of alternative care and transport options, which may include directing them to other services:

5.3.1 Private transport to a clinic, physician's office, or an Emergency Department

5.3.2 Telephone consultation with a physician

Reasonable Force and Restraints

Only use minimum force/restraint necessary for safety

Always ensure scene safety first, and call PD

Restraints:

- Leather or soft only
- Check circulation distal to restraints every 5 minutes
- Do not jeopardize patient medical status (ABC's) or treatment method
    - Never restrain prone
- Handcuffs require PD ride along and cannot be attached to the stretcher

ALCO Restraint Protocol (pg. 115)

1. Patient restraints are to be utilized only when necessary and in those situations where the patient is exhibiting behavior deemed to present danger to him/herself or to the field personnel. When restraints are used:

    1.1 The minimum restraint necessary, to accomplish necessary patient care and safe transportation, should be utilized

    1.2 Circulation to the extremities (distal to the restraints) will be evaluated q 5 minutes

    1.3 Leather or soft restraints, designed specifically for patient restraint, are the only authorized method of restraining patients.

    1.4 The restraints must not be placed in such a way as to preclude evaluation of the patient's medical status (e.g. airway, breathing, circulation) necessary patient care activities, or in any way jeopardize the patient medically

2. If the patient is under arrest and handcuffs are applied by law enforcement officers:

    2.1 The patient will not be cuffed to the stretcher and a law enforcement officer shall accompany the patient in the ambulance, if the handcuffs are to remain applied

    2.2 A law enforcement officer may elect to follow the ambulance in a patrol car to the receiving facility if the patient has been restrained on the gurney using leather or soft restraints

Patient Hold Protocols

WIC 5150 (≤72 hrs): [WIC 5585 for minors]
Emergency assessment required as determined by EMS, PD, or medical/psych staff

WIC 5250 (≤14 days):
Extended treatment requirement determined by psychiatrist

WIC 5270 (≤30 days):
Hold extension initiated by court or psychiatric facility

These protocols disallow a patient from refusing transport, although they may still refuse treatment (ALCO manual pg. 122)

5150 Criteria

Danger to Self (DTS)
Suicidal ideation with intent/plan, active self-harm

Danger to Others (DTO)
Explicit or physical threat to harm others tied to a mental health episode

Gravely Disabled (GD)
Inability to provide for basic personal needs of food, clothing, or shelter due to a mental health disorder

Patients on a 5150 who are not suffering an acute medical/traumatic condition should be transported to John George Pavilion in San Leandro

How to Initialize a 5150

1) Maintain safety and attempt to de-escalate when possible
2) Assess and determine that transport is necessary
3) If the patient refuses transport, call law enforcement or a designated unit to evaluate and execute the 5150
4) Document and keep a copy of the 5150 for pass-off
5) Use reasonable force and restraints if necessary to transport patient

If law enforcement or designated units are unavailable, transport immediately (implied consent) and coordinate paperwork on arrival.

Scenario #1 22 y/o female

Dispatch:

“Respond Code 3 to the main library, second-floor restroom, 22 year old female, chest pain and difficulty breathing. Conscious and breathing. Caller is a friend on scene.”

Scenario #1 22 y/o female

1. Scene Size-up

- Scene safety! Library bathroom is a public space — clear out onlookers
- Pt is on the floor against the wall, knees up, shaking, breathing fast; one friend is kneeling beside her
- No trauma, no vomit, no drug paraphernalia, no obvious life threats
- Identify the friend as your bystander history source — she found the pt and called 911
- Number of patients: one. Additional resources: none yet

Scenario #1 22 y/o female

2. Primary Assessment

- General impression: anxious, crying, hyperventilating — but do not assume this is “just anxiety”
- Mental status: alert, A&O x4, distracted, keeps asking “Am I going to die? Is it my heart?”
- Airway: patent, speaking in short sentences
- Breathing: ~30/min, shallow, regular; lungs clear and equal, no wheezes or stridor
- Circulation: radial pulse strong, rapid, regular; skin pale, cool, diaphoretic; cap refill under 2 sec
- Priority: not an immediate life threat, but chest pain means we keep cardiac on the list

Scenario #1 22 y/o female

3. Secondary Assessment

- Comfort and reassure patient; position of comfort (sitting up); clear the room
- Slow the respiratory rate with coached box breathing — NO paper bag, never “calm down”
- SAMPLE and OPQRST for chest pain (rule out medical): sudden onset 15 min ago, “tight band,” no radiation, 7/10; NKDA; birth control only; no cardiac/asthma/diabetes hx; 3 energy drinks, no sleep, midterm in one hour
- Also reports tingling in both hands and around the mouth, dizziness, sense of doom
- Vitals: BP 138/88, HR 124, RR 30, SpO2 99% RA, BGL 96, pupils PERRL — SpO2 99%: is oxygen indicated? Tingling = hypocapnia, not hypoxia
- Reassess after 5 min: HR 98, RR 18, tingling gone. First episode with chest pain — encourage transport for an ECG

Scenario #2 34 y/o male

Dispatch:

“Respond Code 2 to 1800 Shattuck Ave, Apt 3B — 34 year old male, psychiatric emergency, acting strangely. Caller is the patient’s sister. Law enforcement requested and en route, ETA unknown.”

Scenario #2 34 y/o male

1. Scene Size-up

- Behavioral emergencies are unpredictable and volatile! Stage for law enforcement if anything feels unsafe
- Sister meets you in the hallway: ask about weapons and any history of violence BEFORE entering (none; no violence history)
- Inside: lights off, windows covered in foil, dishes piled up; pt pacing in the far corner, muttering, staring at the air vent
- No weapons visible, no odor of alcohol; pt is unkempt and looks thinner than his photos
- Keep a clear path to the door — never let the pt get between you and the exit
- Don’t assume there is only one patient; confirm the sister is uninjured

Scenario #2 34 y/o male

2. Primary Assessment

- General impression: guarded, pacing, responding to something you can't hear — suspicious but NOT aggressive
- Mental Status Exam: alert; A&O x3 (person, place, approximate date) — believes the EMTs were "sent by the neighbor"
- Responding to internal stimuli: talks back to voices ("I know, I KNOW"), glances over your shoulder
- Airway patent; breathing 18, unlabored, lungs clear
- Circulation: radial 96 strong and regular; skin pink, warm, dry; no bleeding, no trauma
- Approach: 6+ feet, hands visible, low voice, introduce yourself honestly, ask permission before any touch

Scenario #2 34 y/o male

3. Secondary Assessment

- Delegate history to your partner with the sister; be polite and respectful, use active listening and open-ended questions
- OPQRST, AEIOU-TIPS, SAMPLE: BGL 88, SpO2 98%, T 98.6, pupils 4 mm PERRL, no trauma or track marks; NKDA; risperidone 4 mg — stopped 3 wks ago, bottle nearly full; schizophrenia dx at 24, two prior admissions; barely eating for days
- Ask directly about suicidal and homicidal ideation — denies both; no plan to hurt the neighbor
- Vitals: BP 132/84, HR 96, RR 18, SpO2 98% RA. Restraints NOT indicated — not a danger to self or others
- Transport to an ED with psych capability; if he refuses, PD + medical direction re: 5150 (gravely disabled)

Scenario #3 47 y/o male

Dispatch:

"Respond Code 3 to 2900 Cedar St — 47 year old male, suicidal, conscious and breathing. Caller is the patient's brother, who is on scene. Law enforcement dispatched and en route."

Scenario #3 47 y/o male

1. Scene Size-up

- Behavioral emergencies are unpredictable and volatile! Coordinate with law enforcement per protocol
- Brother meets you at the door: “He’s in the back bedroom, he hasn’t done anything yet”
- Ask about weapons BEFORE entering — there is a firearm in the house, locked in a garage safe; pt does not have it
- Be aware of the dangers tied to the patient’s chosen means — look for anything that could be used as a weapon
- Bedroom: dark, quiet; pt sitting on the edge of the bed, hunched, hands in lap; handwritten note on the nightstand
- No blood, no pill bottles visible, no odor of alcohol. From this point on, the pt is never left alone

Scenario #3 47 y/o male

2. Primary Assessment

- General impression: quiet, flat, slow to answer, avoids eye contact — calm is NOT the same as safe
- Mental Status Exam: alert, A&O x4; unkempt, 3–4 days of beard growth; flat affect, long pauses; thoughts organized, no hallucinations
- Airway patent; breathing 16, unlabored, lungs clear and equal
- Circulation: radial 78 strong and regular; skin pink, warm, dry; cap refill under 2 sec
- Control bleeding / check for injury: look at wrists, forearms, neck — nothing found
- Ask directly whether he has taken anything — “No.” Brother confirms he sounded normal on the phone 30 min ago

Scenario #3 47 y/o male

3. Secondary Assessment

- Sit at or below his eye level; redirect the brother so the pt speaks for himself; be polite, respect his privacy
- He only discloses to direct questions: ideation daily for 2 wks, a specific plan, access to means, a note tonight, a goodbye call. No prior attempts. Denies HI. Protective factor: “couldn’t do it to the kids”
- SAMPLE: PCN allergy; lisinopril; sertraline never filled; HTN, depression dx 3 wks ago; father died by suicide; 2–3 beers nightly; job loss, wife moved out
- Vitals: BP 146/92, HR 78, RR 16, SpO2 98% RA, BGL 102. Secure means (firearm, meds); never leave him alone
- Ideation + plan + means = HIGH risk. He cannot refuse transport — PD + medical direction for a 5150; ED for psych evaluation

Kahoot

https://create.kahoot.it/share/neurologic-vs-psychiatric-emergencies-for-emts/2c11e2b4-7de1-4c3d-b601-73059281be2c